Aryan Sharma.
UX/UI & Product Designer

Haven OS

Making the hospital journey
understandable and actionable.

Independent MVP
Product Design Case Study

A shared view of the hospital journey, built around people, information, and what happens next.

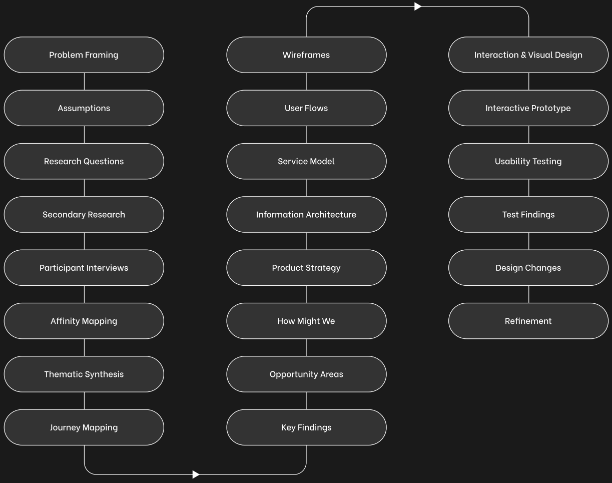
Project Overview

Haven OS is a hospital-first care coordination platform built around the people involved in a hospital stay. It brings together the information, responsibilities, financial and insurance updates, and next steps that are often spread across different teams and systems, giving everyone a clearer view of what's happening and what needs to happen next.

My Role : UX Research & Product Design
Scope : Research to Interactive Prototype

Timeline : 15 Weeks
Type : Independent MVP Product Design

What I Worked Through?

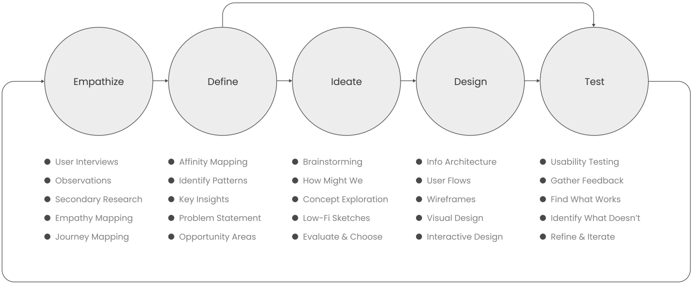


- Problem Framing
- Assumptions
- Research Questions
- Secondary Research
- Participant Interviews
- Affinity Mapping
- Thematic Synthesis
- Journey Mapping
- Key Findings
- Opportunity Areas
- How Might We
- Product Strategy
- Information Architecture
- Service Model
- User Flows
- Wireframes
- Interaction & Visual Design
- Interactive Prototype
- Usability Testing
- Test Findings
- Design Changes
- Refinement

My Design
{Thinking Process}

A human-centered, iterative process to understand people's needs, define the right problem and design meaningful solutions.

Iterate : Learn, refine & improve based on feedback.



Empathize	Define	Ideate	Design	Test
User Interviews	Affinity Mapping	Brainstorming	Info Architecture	Usability Testing
Observations	Identify Patterns	How Might We	User Flows	Gather Feedback
Secondary Research	Key Insights	Concept Exploration	Wireframes	Find What Works
Empathy Mapping	Problem Statement	Low-Fi Sketches	Visual Design	Identify What Doesn't
Journey Mapping	Opportunity Areas	Evaluate & Choose	Interactive Design	Refine & Iterate

Human centered : Always keep people at the center

{Empathize Phase}

What I Wanted To Understand?

Responsibility

Who do they turn to when something changes?

Understanding

What information is difficult to make sense of?

Family

How do different family members stay involved?

Information

How do people receive and keep track of updates?

Operations

Where do hospital teams need to communicate or follow up?

A hospital stay is one

journey. The information supporting it isn't.

A hospital stay involves a lot of people, departments, and moving parts. Everyone has a piece of the picture, but no one always has the whole thing.

Doctor

Diagnosis, treatment plan and clinical updates

"How is treatment progressing?"

Nurse

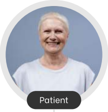
Daily care, observations and on-ground updattes.

"What's the next procedure?"

Family

Stays informed, makes decisions & manages responsibilities.

"What's the latest update?"



Patient

Billing

Generates charges, explains bills and processes payments.

"Whats miscellaneous charges?"

Insurance

Eligibility, authorization and claim processing.

"Are medications covered?"

Reports / Documents

Lab results, scans, prescriptions and discharge instructions

"Where's my recent CT scan?"

Different Information
Different Responsibilities
Different Systems
Different Questions

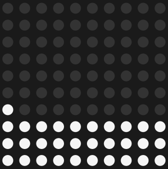
>>>

Everyone Is Working Around The Same Patient, But No One Sees The Whole Picture.

{Secondary Research}
Things That Caught My Attention

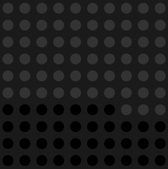
31%

Discharge information was rarely presented as one complete picture.



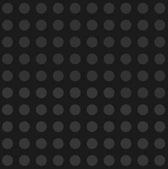
36.84%

Prescription information wasn't always easy to understand.



47%

Most of the times, a fully itemized bill wasn't guaranteed.



Humphries et al. · PLOS ONE · 2020
Das, Kotal & Chatterjee · Indian Journal of Pharmacology · 2025
LocalCircles · World Health Day survey · 7 April 2024

{Qualitative Research}
That gave me a direction.
I wanted to see how this played out for people inside the hospital.

08

Participants

Patients, family, doctors, nurses, billing and insurance staff

03

Hospitals

SMVD Narayana, Batra Hospital, Ujala Cygnus JK Medicity

05

Perspectives

Patient, family, clinical, operations and insurance

Hospital	Patient & Family	Clinical	Operations
SMVD Narayana Katra	{P01} Family Caregiver Insured father hospitalized {P02} Patient Planned surgery with employer insurance	—	—
Batra Hospital Jammu	{P03} Spouse/Caregiver 12 Days hospitalization mostly self-paid {P04} Remote Family Member Following care from Delhi	{P05} Doctor Clinical workflow & communication {P06} Ward Nurse Care Updated & family communication	—
Ujala Cygnus JK Medicity Jammu	—	—	{P07} Billing Staff Charges and explanation {P08} Insurance Staff Authorization & documentation

{My Approach} What I Wanted To Understand?

The method of semi-structured interviews was applied to uncover patient, family, and hospital staff experiences while being in hospital. My interest was in the actual experiences, behaviour, workaround, and moments of confusion, rather than asking people to design the product.

Glimpse of questions (Translation of conversation conducted in local language)	Focus
How does a hospital stay unfold from admission to discharge?	The Journey
How do people receive, keep, and share updates?	Information
Who do they turn to when they need help or something changes?	Responsibility
What is difficult to understand around care, billing, and insurance?	Understanding & Money
How do people divide responsibilities, handle gaps, and figure out what happens next?	Coordination

“How was your past hospital experience?”

“How did you keep track of it”

“How did you communicate with the doctor”

Interview Structure

01

Open questions

Started with broad, open-ended questions to understand their experience in their own worlds

02

Follow-up questions

Asked follow-up questions to explore specific situations, decisions, and possible touchpoints

03

Probing for behaviour

Dived deeper into actions, context and workarounds to understand actual behavior rather than expected behavior

04

Synthesis

Captured key points from each conversation and grouped them into patterns, workarounds, and unmet needs.

A Few Things Stayed With Me

“I usually asked whoever was around. Sometimes I'd ask two people because I wasn't sure who was supposed to answer.”

Family caregiver
Translated and paraphrased from interview notes.

“The bill was visible, but the individual charges weren't always clear.”

Patient
Translated and paraphrased from interview notes.

“The person at home wanted to know how the patient was doing and whether there was anything they needed to pay.”

Remote Family Member
Translated and paraphrased from interview notes.

What Kept Coming Up?

Across 8 semi-structed interviews with patients, family members and hospital staff, a few themes appeared repeatedly. These numbers show how many participants talked about each theme, giving a sense of what was most common in our conversation.

6 out of 8

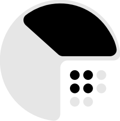
Participants described getting information through multiple people, documents, channels, or systems.

6 out of 8

Participants were often unsure what would happen next, or what they were expected to do.

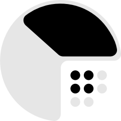
5 out of 8

Participants found some information difficult to understand without enough context or explanation.



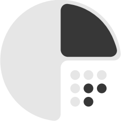
4 out of 8

Participants found bills, insurance coverage, and changing financial information difficult to track and understand.



4 out of 8

Participants described information moving between departments, systems, and people during different stages of the journey.



3 out of 8

Participants were sometimes unsure who to approach, or who was responsible for the next step.

Research Evidence	Interpretation	Design Opportunity
Participants sought updates from multiple people	Information access depends on finding the right person	Consolidated journey updates
Participants were unsure what happened next	Status alone doesn't explain the next action	A clear next-step section
Financial information was difficult to follow	Amounts need context, not just totals	Payment status, pending balance and insurance state together

{Empathy Map}
Different Roles, Different Realities

	Patient/Family	Clinical	Billing/Insurance

Thinks	What's happening? Who should I ask? What do I need to do next?	What needs my attention? What has changed? What still needs explanation?	What is pending? Are the documents complete? Who needs to act next?
Feels	Uncertain Overwhelmed Reassured when informed	Responsible Time-pressured Cautious about communication	Pressured Frustrated by missing information Focused on resolution
Says	"I usually asked whoever was around.", "The bill was visible, but not always clear."	"Families often ask about progress and timing."	"Charges and approvals sometimes need explanation."
Does	Asks multiple people Keeps documents Relays updates to family	Updates families Coordinates with other teams Answers repeated questions	Checks documents Follows up with departments Explains charges/status
Needs	Clear updates Context Visible ownership Next steps	Relevant context Clear handoffs Less repetitive communication	Complete information Status visibility Clear ownership Efficient handoffs

This empathy map synthesizes recurring themes across 8 interviews. Statements are translated and paraphrased from handwritten notes and are not verbatim quotes.

{Key Insights}
User Study Takeaways

Information exists, but the journey is fragmented

People described getting information from doctors, nurses, relatives, documents, reports, and different hospital functions.

Knowing what happened isn't enough

Across care, billing, insurance, and discharge, people also needed to understand who was responsible and what happened next.

The family often becomes the bridge

Different family members may handle updates, documents, payments, or communication with relatives, so the "user" is often a small network rather than one person.

{Current State Journey}
Hospital Journey, From The Inside

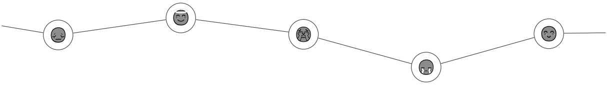
Based on 8 qualitative interviews, this journey shows how information, people and responsibilities change across the hospital stay, and where friction commonly appears.

01	02	03	04	05
Admission	**Treatment**	**Communication**	**Finance & Insurance**	**Discharge**
Many people start generating information.	Information changes continuously.	Families often relay updates between one another.	Costs, approvals and responsibility can become unclear.	Information turns into things the patient or family must manage at home.

Key Actions

Admission	Treatment	Communication	Finance & Insurance	Discharge
● Shares personal & medical details	● Undergoes tests and procedures	● Asks for updates from staff	● Review bills and estimates	● Receives discharge summary
● Submits documents	● Receives ongoing updates	● Shares information with family	● Shares documents for insurance	● Collects final bills and reports
● Completes initial forms and payments	● Multiple teams get involved	● Repeats the same question to different people	● Follows up on approval and coverage	● Understands medications and follow-up care

Emotional Journey



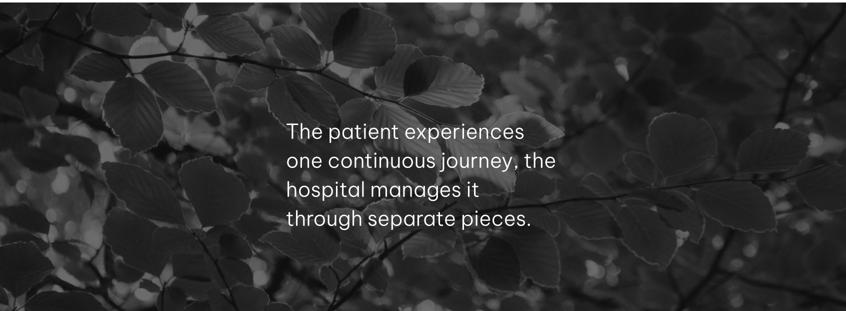
Touch Points

Admission	Treatment	Communication	Finance & Insurance	Discharge
Reception	Doctors	Phone calls	Billing desk	Discharge desk
Admission desk	Nurses	In-person updates	Insurance team	Pharmacy
ID & insurance verification	Test and diagnostics	Whatsapp messages	Approvals & documents	Discharge summary
	Ward Staff	Multiple staff members	Payment counters	Follow-up appointment

Key Pain Points

Admission	Treatment	Communication	Finance & Insurance	Discharge
Unclear what documents are needed & what happens next.	Information changes frequently and is hard to keep track of.	Have to ask multiple people and repeat the same questions.	Bill is visible, but not always clear. Insurance process is difficult to understand.	Still unsure about next steps, medications, or who to contact after leaving.

Anxious & uncertain
Hopeful & overwhelmed
Inconsistent & confused
Stressed & frustrated
Relieved, but still has many questions



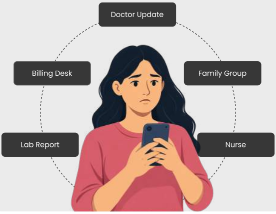
The patient experiences one continuous journey, the hospital manages it through separate pieces.

{Define Phase}

The Problem Wasn’t Lack Of Information, It Was What Happened Between It

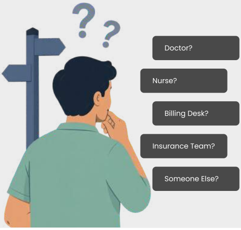
Information was scattered

Updates lived across doctors, nurses, documents, systems, phone calls and whatsapp conversations.


Doctor Update
Billing Desk
Family Group
Lab Report
Nurse

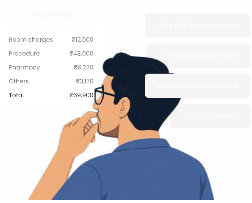
Responsibility was unclear

People could often see the issue, but not always who owned it or who they should approach.


Doctor?
Nurse?
Billing Desk?
Insurance Team?
Someone Else?

Visibility didn’t always mean understanding

Seeing a bill, update or clinical detail was not always enough to understand what changed and what it meant.



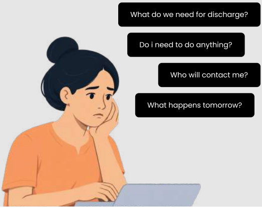
Hospital Bill

Room charges	₹12,500
Procedure	₹48,000
Pharmacy	₹6,230
Others	₹3,170
Total	₹69,900

Why did this increase?
Do i need to pay now?
What does this mean?
Is this covered?

The next action was often missing

People were repeatedly trying to work out what happens next, what they needed to do, and who needed to act.


What do we need for discharge?
Do i need to do anything?
Who will contact me?
What happens tomorrow?

{Lightweight Personas}
Who Needs Haven OS?

Patient	Family	Doctor	Nurse	Billing/Insurance
Context : Receives Care	**Context :** Coordinates Support	**Context :** Manages diagnosis/treatment	**Context :** Coordinates day to day care	**Context :** Handles financial flow
Goal : Understand treatment, recovery and next steps	**Goal :** Get real time updates, manage responsibilities, stay aligned with family	**Goal :** Access key patient information, communicate plans clearly	**Goal :** Track care updates, follow up on tasks, communicate changes	**Goal :** Verify eligibility, process claims, explains bills clearrly
Friction : Information is medical and hard to understand, relies on others for updates	**Friction :** Has to ask multiple people, information is scattered, hard to track changes	**Friction :** Information is spread across systems, repeated questions from family	**Friction :** Relies on calls, in-person updates, unclear ownership for certain issues	**Friction :** Missing or incomplete documents, back and forth with other teams

{User Journey Mapping}
What Does Each Role Need From The Same Journey?

	Admission	Diagnosis	Treatment	Billing & Insurance	Discharge
Patient	Clear explanation of what to expect and what's happening	Understand diagnosis and treatment options in simple terms	Know how treatment is progressing and what comes next	Understand charges, insurance coverage and out of pocket cost	Clear discharge instructions and follow-up plan
Family Caregiver	Timely updates about admission and initial condition	Clear information about diagnosis, treatment plan and risks.	Regular updates on condition and upcoming procedures	Transparent breakdown of bills and what is covered by insurance	Guidance on post-discharge care, medications and next appontments.
Doctor	Access to complete patient history and relevant documents	All test results and reports in one place for faster decision making	Real-time view of patient status, notes and team updates	Clinical justification and documentation for procedures	Ensure discharge plan is clear and shared with patient in family
Nurse	Patient details, care plan and any special needs	Clear orders and updated treatment plan	Up-to-date task list, medication schedule and patient status	Awareness of insurance approval status for procedures	Checklist of discharge tasks, patient education & follow-up

Billing/Insurance	Patient and insurance details for verification	Information about planned procedures and coverage	Updates on additional procedures, changes in treatment plan	Generate itemized bills, process insurance claims and track payments	Finalize billing, share documents and close claims
Key Needs	Clear information on next steps and required documents	Understand results, treatment options and plan	Regular updates on progress and changes in care	Transparent, easy to understand charges and insurance coverage	Clear discharge plan, medications and follow-up care
Pain Points	Repeats the same information multiple times	Information is spread across different people and systems	Unclear ownership of updates and next steps	Bills are complex and not fully itemized	Discharge instructions and follow-ups are not presented as one complete picture

{SWOT Analysis} Understanding The Competitive Landscape

Competitors	Practo	Healthray
Strengths	Registration, billing, labs, insurance/TPA and patient-portal options.	Broad OPD/IPD, EMR, pharmacy, laboratory, billing and TPA workflows, with ABHA support.
Weakness	Public feature pages emphasise operational breadth; the depth of its patient-and-family coordination experience needs further review.	The breadth of the suite is clear, but the dedicated patient/family experience needs closer product-level assessment.
Opportunities	Make outstanding tasks, responsibility and the patient's next step clearer across workflows.	Further connect clinical and administrative updates into role-specific patient and family experiences.
Threats	Competing hospital-management suites can offer similar workflows through an existing vendor.	Competing all-in-one HMS platforms may weaken its differentiation.

{Key Opportunity Areas}
Where Should
Haven OS Intervene?

Create a shared patient journey

Bring together key updates, care events, documents and next steps in one place for everyone involved.

Make responsibility visible

Show who owns each task or question, who can provide an update and what needs to happen next.

Turn financial information into a clear state

Connect current charges, what changed, insurance contribution and required action in a simple, transparent way

{How Might I}
What Should Haven OS
Actually Help People Do?

It's difficult to understand what's happening and what comes next.

Discharge information is rarely presented as one complete picture.

People have to ask multiple people to get a complete update.

Important updates and documents are easy to miss.

Information is spread across different people and systems.

How might I give everyone a shared view of the hospital journey without overwhelming them with information?

People often don't know who to ask when something changes

It's unclear who is responsible for updates and next steps

Family members have to follow-up multiple times every day

Ownership of information can be unclear across different teams.

People wants clarity on what needs to be done and by whom

How might I make responsibility and the next action clear when something needs and answer, an update or a decision?

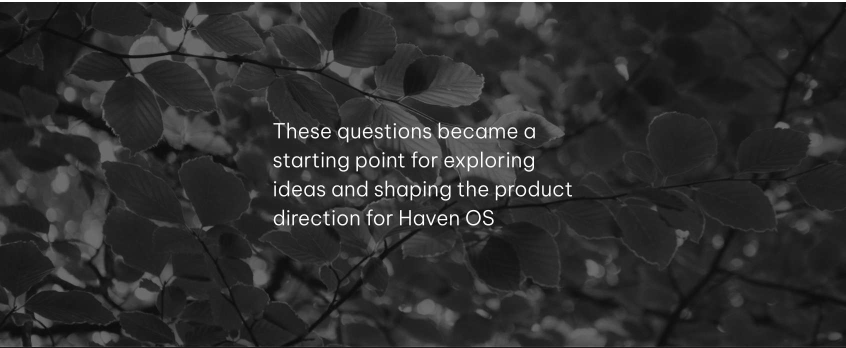
Bills are complex and not always easy to understand

People are unsure what is covered and what they have to pay

Insurance status and approvals are not clear in real time

How might I make financial and insurance information easier to understand and act on?

People want a simple breakdown and clear next steps for payment



These questions became a starting point for exploring ideas and shaping the product direction for Haven OS

Defining The MVP

Haven OS has a wide potential, but for the first version, I focused on the core problems that create the most impact in a hospital journey. The MVP is designed to deliver a clear, useful and achievable experience.

✓ In scope

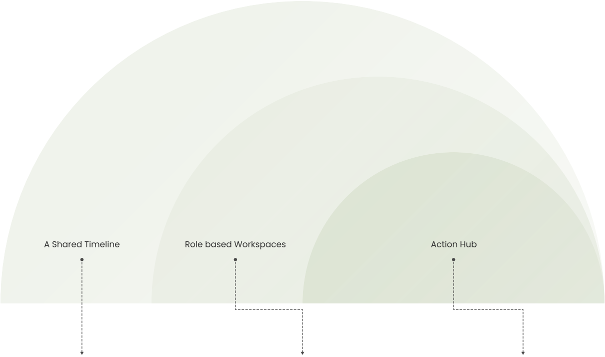
- Shared patient journey
- Ownership and next actions
- Financial and insurance state
- Important documents
- Relevant notifications
- Role based experiences

☒ Out of scope

- Replacing hospital systems
- Diagnosis or treatment decisions
- Real-time clinical tools
- Complete hospiital ERP
- Prescription ordering
- Long-term health management

{Ideate Phase}
Exploring The Solution Space

I explored different ways Haven OS could bring clarity and coordination to the hospital journey. Each direction addressed parts of the problem, but also had limitations. Exploring multiple approaches helped me understand what to combine, what to avoid and what to take forward.



A Shared Timeline

A single, chronological view of everything that happens during the hospital stay.

- Gives a clear holistic view
- Easy to follow for patient & families
- Brings different events together

Role based Workspaces

Tailored experiences for different people involved in the same hospital journey

- Relevant information for each role
- Reduces cognitive load
- Feels personalized and actionable

Action Hub

Focused on open tasks, ownership and next steps to move the hospital journey forward

- Highlights what needs attention
- Shows ownership and next steps
- Helps different teams stay aligned

- Risk of becoming a long list
- Doesn't highlight ownership
- Can be overwhelming & complex

- Risk of silos b/w different views
- Harder to maintain consistency
- Doesn't fully solve coordination problem

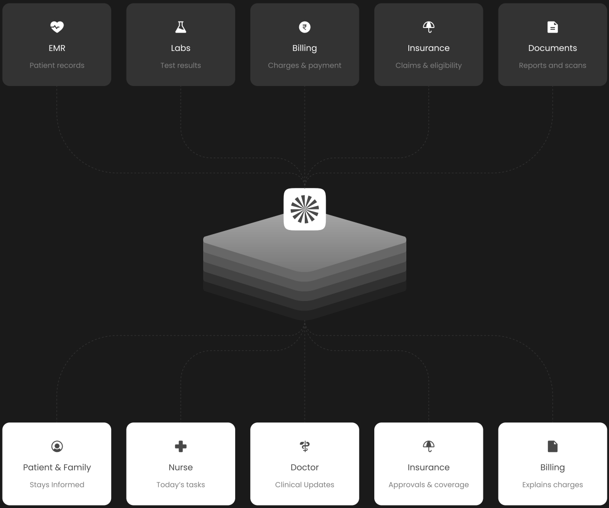
- Doesn't show the full context easily
- Requires strong underlying info structure
- May feel task heavy for patients

From Multiple Directions, To One Cohesive System

I chose to build one connected system underneath, with different experiences on top acting as a shared coordination layer for the hospital journey

Existing Hospital Systems

Haven OS works with the tools hospitals already use



EMR
Patient records

Labs
Test results

Billing
Charges & payment

Insurance
Claims & eligibility

Documents
Reports and scans

Patient & Family
Stays Informed

Nurse
Today's tasks

Doctor
Clinical Updates

Insurance
Approvals & coverage

Billing
Explains charges

A Shared Connection Layer

It connects information from different systems, adds context, ownership and next steps, and keeps everyone aligned around the same patient journey.

{Structuring The Experience} Different Roles Need Different Views. They Don't Need Different Systems.

I structured Haven OS around the work each role needs to do, while keeping the underlying information consistent across the product.

Hospital Staff

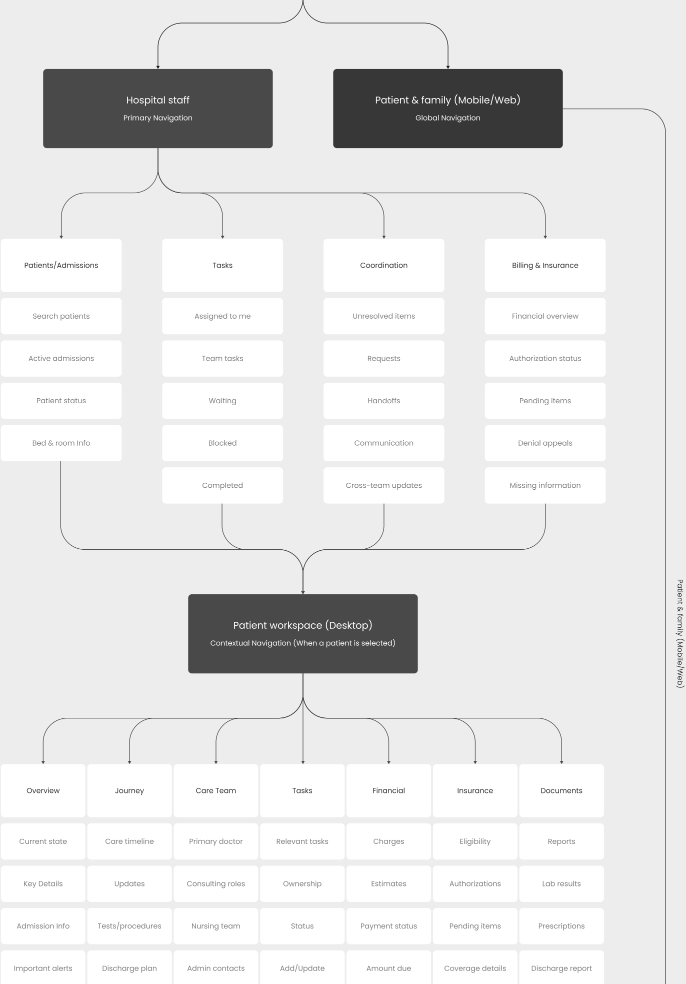
- Care Team
- Patients
- Billing & Insurance
- Documents
- Tasks

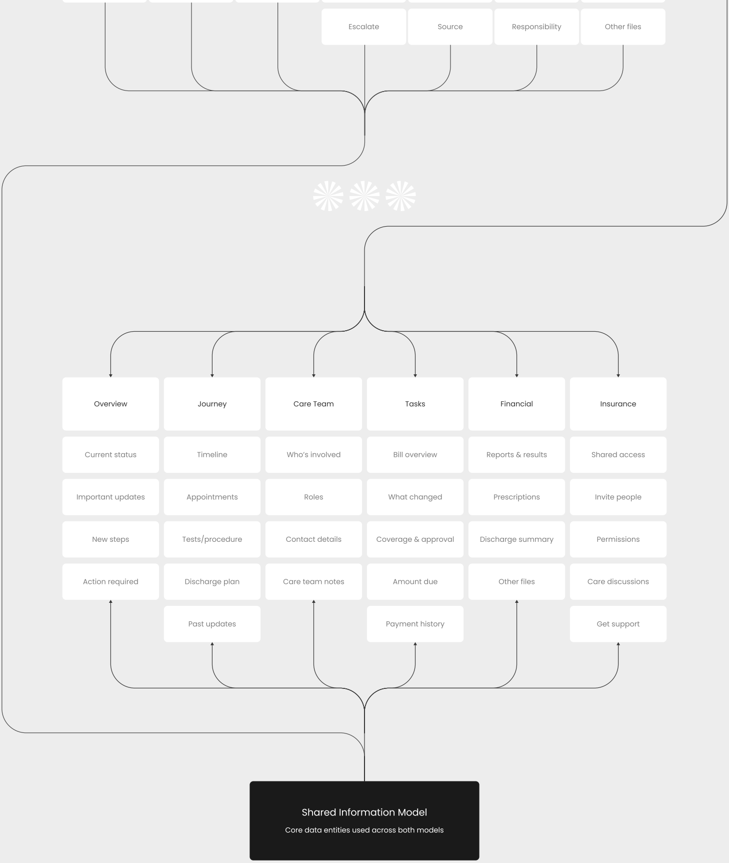
Patient & Family

- Today
- Bills & Insurance
- Care Team
- Journey
- Family
- Documents

Haven OS

Information Architecture


Hospital staff
Primary Navigation
Patient & family (Mobile/Web)
Global Navigation
Patients/Admissions
Search patients
Active admissions
Patient status
Bed & room Info
Tasks
Assigned to me
Team tasks
Waiting
Blocked
Completed
Coordination
Unresolved items
Requests
Handoffs
Communication
Cross-team updates
Billing & Insurance
Financial overview
Authorization status
Pending items
Denial appeals
Missing information
Patient workspace (Desktop)
Contextual Navigation (When a patient is selected)
Overview
Current state
Key Details
Admission Info
Important alerts
Journey
Care timeline
Updates
Tests/procedures
Discharge plan
Care Team
Primary doctor
Consulting roles
Nursing team
Admin contacts
Tasks
Relevant tasks
Ownership
Status
Add/Update
Financial
Charges
Estimates
Payment status
Amount due
Insurance
Eligibility
Authorizations
Pending items
Coverage details
Documents
Reports
Lab results
Prescriptions
Discharge report
Patient & family (Mobile/Web)



Escalate
Source
Responsibility
Other files

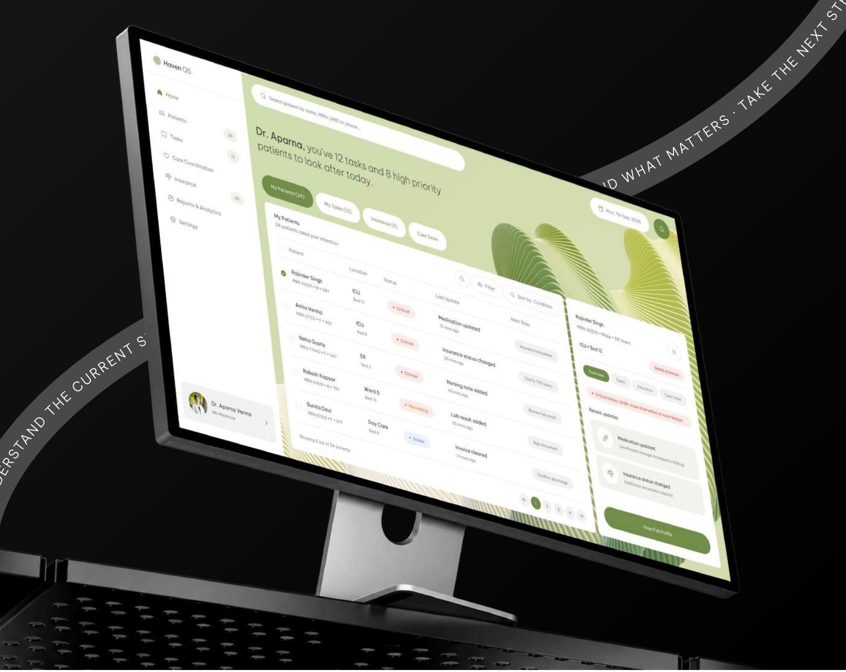
Overview	Journey	Care Team	Tasks	Financial	Insurance
Current status	Timeline	Who’s involved	Bill overview	Reports & results	Shared access
Important updates	Appointments	Roles	What changed	Prescriptions	Invite people
New steps	Tests/procedure	Contact details	Coverage & approval	Discharge summary	Permissions
Action required	Discharge plan	Care team notes	Amount due	Other files	Care discussions
	Past updates		Payment history		Get support

Shared Information Model

Core data entities used across both models

{Design Phase - Hospital} A Clear View Of What Needs Attention.

Once the structure was clear, I started designing the screens and figuring out what information was needed to be front and center.


ERSTAND THE CURRENT S
D WHAT MATTERS · TAKE THE NEXT STE

Haven OS
Home
Patients
Tasks 24
Care Coordination 12
Insurance
Reports & Analytics 05
Settings
Search patient by name, MRN, UHID or phone...
Mon, 7th Sep, 2026
Dr. Aparna Verma
MD Medicine

Dr. Aparna, you've 12 tasks and 8 high priority patients to look after today.

My Patients (24)
My Tasks (12)
Insurance (5)
Care Team
Filter
Sort by : Condition

My Patients

24 patients need your attention

Patient	Location	Status	Last Update	Next Step
Rajinder Singh MRN 48291 • M • 58Y	ICU Bed 12	• Critical	Medication updated 10 mins ago	Insurance document
Anita Verma MRN 57321 • F • 46Y	ICU Bed 8	• Critical	Insurance status changed 28 mins ago	Clarify TPA query
Neha Gupta MRN 77442 • F • 34Y	ER Bed 2	• Critical	Nursing note added 45 mins ago	Review lab result
Rakesh Kapoor MRN 61109 • M • 70Y	Ward 5 Bed 15	• Recovering	Lab result added 55 mins ago	Sign document
Sunita Devi MRN 61109 • F • 61Y	Day Care Bed 6	• Stable	Invoice cleared 3 hours ago	Confirm discharge

Showing 5 out of 24 patients

Rajinder Singh

MRN 48291 • Male • 58 Years

ICU • Bed 12

Needs attention
Overview
Tasks
Insurance
Care Team

• Critical status. Under close observation of nurse Neelam

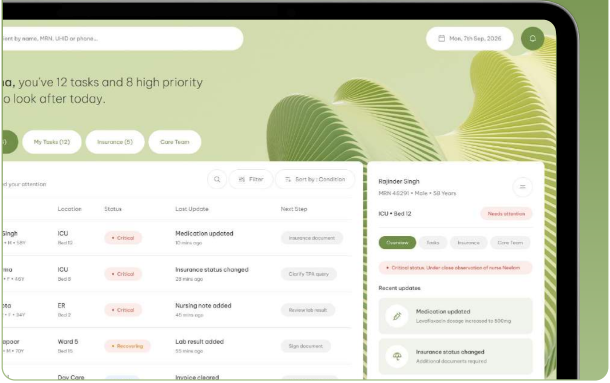
Recent updates

Medication updated
Levofloxacin dosage increased to 500mg

Insurance status changed
Additional documents required

View Full Profile

The desktop {Home screen} brings patient status, recent updates, and next steps into one view, helping staff identify what needs attention without piecing information together across different systems.


ient by name, MRN, UHID or phone...
Mon, 7th Sep, 2026
My Tasks (12)
Insurance (5)
Care Team
Filter
Sort by : Condition
Overview
Tasks
Insurance
Care Team

ıa, you've 12 tasks and 8 high priority
o look after today.

d your attention

	Location	Status	Last Update	Next Step
Singh • M • 58Y	ICU Bed 12	• Critical	Medication updated 10 mins ago	Insurance document
ma • F • 46Y	ICU Bed 8	• Critical	Insurance status changed 28 mins ago	Clarify TPA query
ota • F • 34Y	ER Bed 2	• Critical	Nursing note added 45 mins ago	Review lab result
apoor • M • 70Y	Ward 5 Bed 15	• Recovering	Lab result added 55 mins ago	Sign document
	Day Care		Invoice cleared	

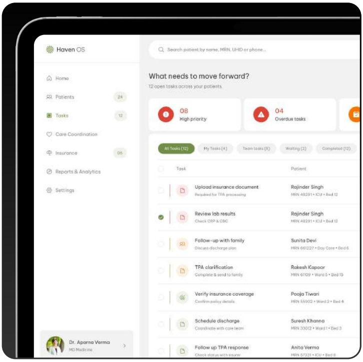
Rajinder Singh
MRN 48291 • Male • 58 Years
ICU • Bed 12
Needs attention

• Critical status. Under close observation of nurse Neelam

Recent updates

Medication updated
Levofloxacin dosage increased to 500mg

Insurance status changed
Additional documents required

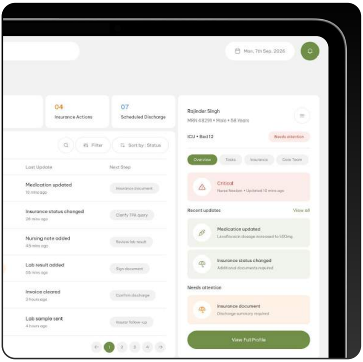

Haven OS
Search patient by name, MRN, UHID or phone...
Home
Patients
24
Tasks
12
Care Coordination
Insurance
05
Reports & Analytics
Settings
All Tasks (12)
My Tasks (4)
Team tasks (8)
Waiting (2)
Completed (12)
Dr. Aparna Verma
MD Medicine

What needs to move forward?
12 open tasks across your patients

08 High priority
04 Overdue tasks

Task	Patient
Upload insurance document Required for TPA processing	Rajinder Singh MRN 48291 • ICU • Bed 12
Review lab results Check CRP & CBC	Rajinder Singh MRN 48291 • ICU • Bed 12
Follow-up with family Discuss discharge plan	Sunita Devi MRN 661227 • Day Care • Bed 6
TPA clarification Complete & send to family	Rakesh Kapoor MRN 61109 • Ward 5 • Bed 15
Verify insurance coverage Confirm policy details	Pooja Tiwari MRN 55902 • Ward 2 • Bed 4
Schedule discharge Coordinate with care team	Suresh Khanna MRN 33012 • Ward 1 • Bed 3
Follow up TPA response Check status with insurer	Anita Verma MRN 57321 • ICU • Bed 8

The {Tasks screen} makes ownership and status clear, helping staff track progress and understand what needs to happen next.


Mon, 7th Sep, 2026
Filter
Sort by : Status
Overview
Tasks
Insurance
Care Team
View all
View Full Profile
1
2
3
4

04 Insurance Actions
07 Scheduled Discharge

Last Update	Next Step
Medication updated 10 mins ago	Insurance document
Insurance status changed 28 mins ago	Clarify TPA query
Nursing note added 45 mins ago	Review lab result
Lab result added 55 mins ago	Sign document
Invoice cleared 3 hours ago	Confirm discharge
Lab sample sent 4 hours ago	Insurer follow-up

Rajinder Singh
MRN 48291 • Male • 58 Years
ICU • Bed 12
Needs attention

Critical
Nurse Neelam • Updated 10 mins ago

Recent updates

Medication updated
Levofloxacin dosage increased to 500mg

Insurance status changed
Additional documents required

Needs attention

Insurance document
Discharge summary required

The {Patients screen} brings patient status, recent updates, and next steps together, making it easier to see who needs attention.

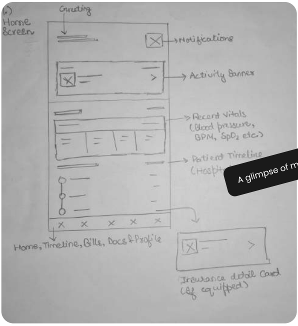
Want to explore the full set of screens or discuss the project further? Reach me at coreux.aryan@gmail.com

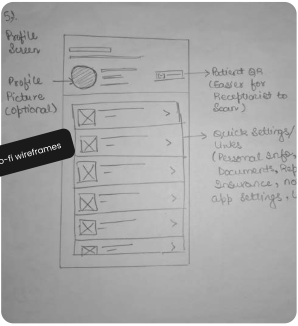
Haven OS
Home
Patients
Tasks
Care Coordination
Insurance
Reports & Analytics
Settings
Dr. Aparna Verma
Insurance cases

Patient	Insurance Provider	Request Type	Last Update	Status	Next Action
Rajinder Singh	Star Health	Cashless approval	Today	Under review	None
Neha Gupta	HDFC Ergo	Pre authorization	Today	Documents needed	Upload documents
Sunita Devi	TATA AIG	Pre authorization	Today	Documents needed	Upload documents
Rakesh Kapoor	ICICI Lombard	Claim settlement	Today	Action required	[illegible]
Pooja Tiwari	ICICI Lombard	Claim settlement	Today	Action required	[illegible]
Neha Gupta	NA	Add-on approval	Yesterday	Under review	None
Sunita Devi	Bajaj Allianz	Claim settlement	Yesterday	Approved	None
Rakesh Kapoor	Star Health	Cashless approval	Wed, 9 Sep	Rejected	[illegible]

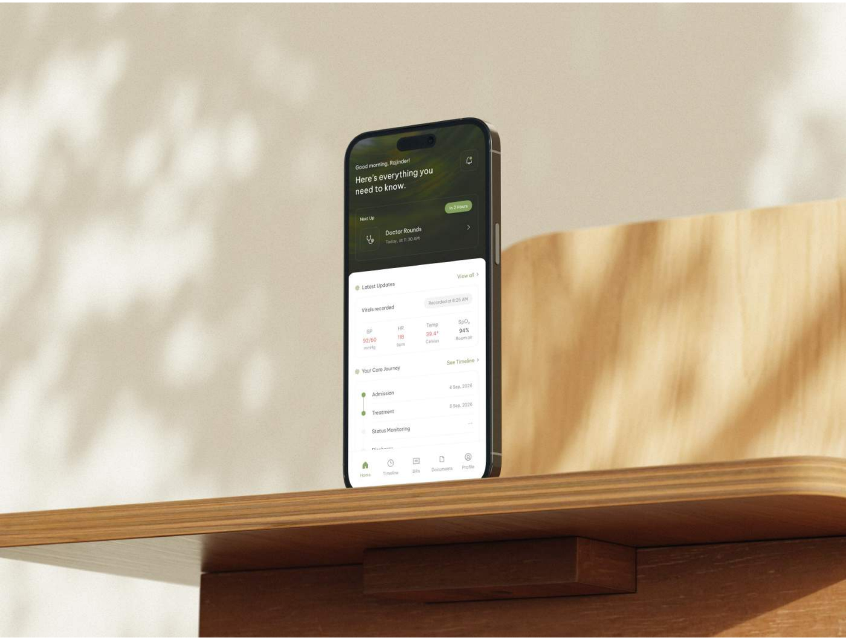
{Design Phase - Patient And Families}
Complete Transparency

Patients & families get a clearer view of the hospital stay, with timely updates, care journey progress, insurance status & important documents in one place.

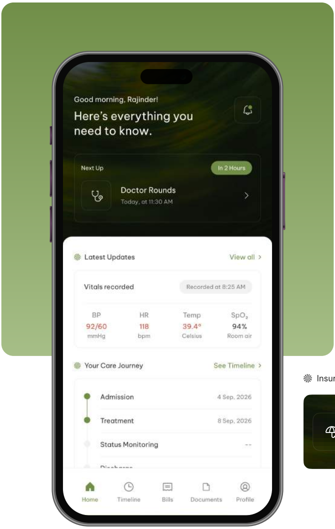

1.)
Home Screen
Greeting
Notifications
Activity Banner
Recent Vitals (Blood pressure, BPM, SpO₂ etc.)
Patient Timeline (Hospit
Home, Timeline, Bills, Docs & Profile
Insurance detail Card (If equipped)


5.).
Profile Screen
Profile Picture (optional)
Patient QR (Easier for Receptionist to Scan)
Quick Settings/ Links (Personal Info, Documents, Rep Insurance, no app Settings,

A glimpse of my lo-fi wireframes


Good morning, Rajinder!
Here's everything you need to know.
In 2 Hours
Next Up
Doctor Rounds
Today, at 11:30 AM
Latest Updates
View all
Vitals recorded
Recorded at 8:25 AM
BP
92/60
mmHg
HR
118
bpm
Temp
39.4°
Celsius
SpO₂
94%
Room air
Your Care Journey
See Timeline
Admission
4 Sep, 2026
Treatment
8 Sep, 2026
Status Monitoring

Home
Timeline
Bills
Documents
Profile


Good morning, Rajinder!

Here's everything you need to know.

Next Up — In 2 Hours

Doctor Rounds
Today, at 11:30 AM

Latest Updates — View all

Vitals recorded — Recorded at 8:25 AM

BP	HR	Temp	SpO_2
92/60	118	39.4°	94%
mmHg	bpm	Celsius	Room air

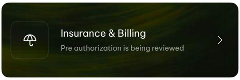
Your Care Journey — See Timeline

Admission	4 Sep, 2026
Treatment	8 Sep, 2026
Status Monitoring	--

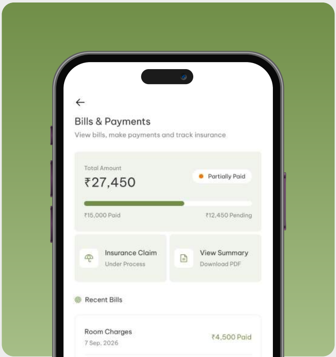
Home
Timeline
Bills
Documents
Profile

The mobile {Home screen} brings recent updates, upcoming events, and important information into one place, helping patients stay informed and understand what happens next during their hospital stay.

Insurance Status


Insurance & Billing
Pre authorization is being reviewed

The {Bills screen} brings charges, payments, and insurance updates together, helping patients understand what's been paid, what's pending, and what needs attention.


Bills & Payments
View bills, make payments and track insurance

Total Amount
₹27,450
Partially Paid

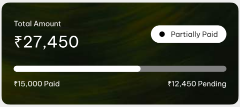
₹15,000 Paid — ₹12,450 Pending

Insurance Claim
Under Process

View Summary
Download PDF

Recent Bills

Room Charges
7 Sep, 2026 — ₹4,500 Paid

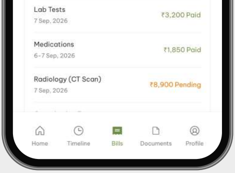

Total Amount

₹27,450

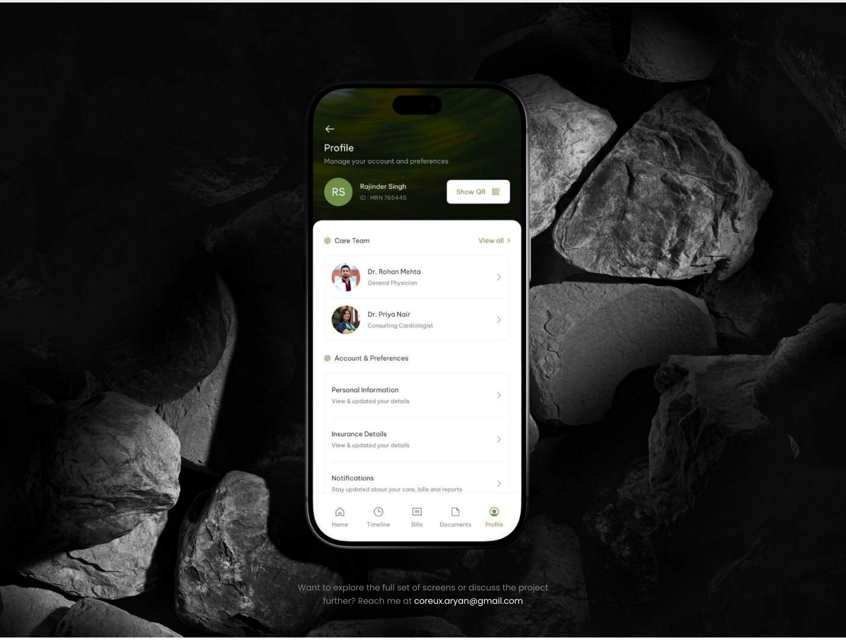
● Partially Paid

₹15,000 Paid

₹12,450 Pending


Lab Tests 7 Sep, 2026	₹3,200 Paid
Medications 6-7 Sep, 2026	₹1,850 Paid
Radiology (CT Scan) 7 Sep, 2026	₹8,900 Pending

Home
Timeline
Bills
Documents
Profile


Profile

Manage your account and preferences

RS Rajinder Singh

ID : MRN 765445

Show QR

Care Team

View all >

Dr. Rohan Mehta

General Physician

Dr. Priya Nair

Consulting Cardiologist

Account & Preferences

Personal Information

View & updated your details

Insurance Details

View & updated your details

Notifications

Stay updated about your care, bills and reports

Home
Timeline
Bills
Documents
Profile

Want to explore the full set of screens or discuss the project further? Reach me at coreux.aryan@gmail.com

{Test Phase}
Testing The Patient/Family App In The Real World

I tested the Haven OS patient app through quick, unmoderated guerilla tests at my run club and async feedback with my design community on whatsapp. The goal was to understand if the core flows felt clear and intuitive for the first time.

06

Participants from run club

Runners, elders, retired professionals, adults and fitness enthusiasts with age group 16-60

05

Participants from UX community

Designers & product folks from my whatsapp community (IITans, Ex-Accenture, Ex-Synechron)

What Participants Were Asked For?

Participants were given a few core tasks to complete on the figma prototype while sharing their thoughts out loud or over chat.

Check current status

Open the app and find the next doctor visit and current status

Explore the timeline

Find recent updates and understand what happened so far

View Bills

Locate the total amount, payments made and pending balance.

Find a report

Locate a lab report or discharge summary in the documents section

What Worked And What Didn’t Work?

Overall, the feedback was positive. Participants found the app clean, easy to navigate and relevant to their needs. The UX community pointed out a few mental model issues, which helped me refine the design further.

01

02

03

What Worked

Easy to find patient status, next action, reports, documents, billing clarity, dedicated care team details and overall hospital journey with on time to time updates history.

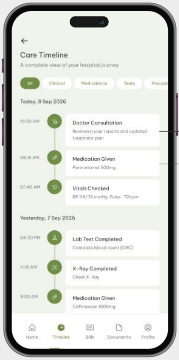
What needs imrovement

Unclear task ownership. When i asked questions about clarity in the timeline, most of the users were satisfied, however, a few expressed a feeling a slight confusion, that they understand what steps are there in the timeline, however, they need to know how conducted those procedures or tasks, that'll help them connect with the responsible person in case of any need or questions.

What didn't work

All the participants in my design community disagreed with my icons usage in the navbar. They questioned my thought process for using dollar icon for bills if the app is primarily for Indian users and the timeline icon which looked more like recent history. I changed them and took feedback again.

Before


All
Clinical
Medications
Tests
Procedu
Home
Timeline
Bills
Documents
Profile

Care Timeline

A complete view of your hospital journey

Today, 8 Sep 2026

10:30 AM — Doctor Consultation
Reviewed your reports and updated treatment plan

08:15 AM — Medication Given
Paracetamol 500mg

07:45 AM — Vitals Checked
BP 118/76 mmHg, Pulse : 72bpm

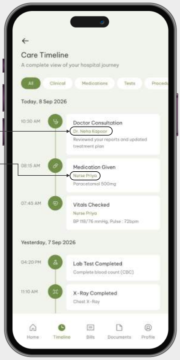
Yesterday, 7 Sep 2026

04:20 PM — Lab Test Completed
Complete blood count (CBC)

11:10 AM — X-Ray Completed
Chest X-Ray

9:00 AM — Medication Given
Ceftriaxone 1000mg

After


All
Clinical
Medications
Tests
Procedu
Home
Timeline
Bills
Documents
Profile

Care Timeline

A complete view of your hospital journey

Today, 8 Sep 2026

10:30 AM — Doctor Consultation
Dr. Neha Kapoor
Reviewed your reports and updated treatment plan

08:15 AM — Medication Given
Nurse Priya
Paracetamol 500mg

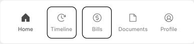
07:45 AM — Vitals Checked
Nurse Priya
BP 118/76 mmHg, Pulse : 72bpm

Yesterday, 7 Sep 2026

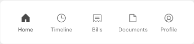
04:20 PM — Lab Test Completed
Complete blood count (CBC)

11:10 AM — X-Ray Completed
Chest X-Ray

Before

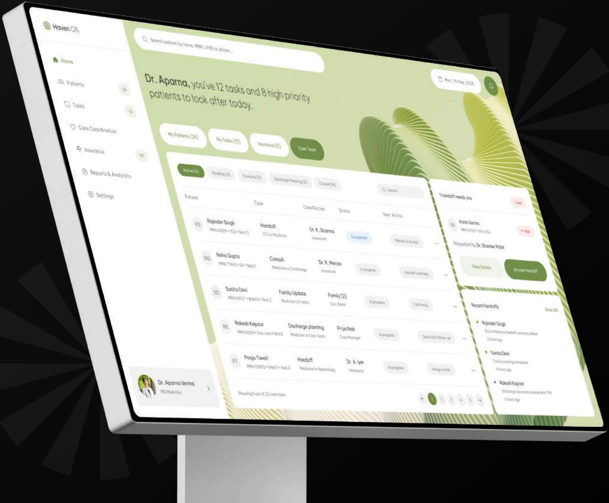

Home
Timeline
Bills
Documents
Profile

After


Home
Timeline
Bills
Documents
Profile

Better Design Starts With Understanding What People Need To Know.

This project reinforced that bringing information together is only part of the problem. People also need context, clear responsibility, and an understandable next step.


Haven OS
Search patient by name, MRN, UHID or phone...
Home
Patients
24
Tasks
12
Care Coordination
Insurance
05
Reports & Analytics
Settings
Mon, 7th Sep, 2026
My Patients (24)
My Tasks (12)
Insurance (5)
Care Team
Active (5)
Pending (4)
Consults (3)
Discharge Planning (2)
Closed (18)
Search
Dr. Aparna Verma
MD Medicine

Dr. Aparna, you've 12 tasks and 8 high priority patients to look after today.

Patient	Type	Care Partner	Status	Next Action
RS **Rajinder Singh** MRN 48291 • ICU • Bed 12	**Handoff** ICU to Medicine	**Dr. K. Sharma** Intensivist	Completed	Review & accept
NG **Neha Gupta** MRN 77443 • ER • Bed 2	**Consult** Medicine to Cardiology	**Dr. R. Menon** Intensivist	In progress	Handoff summary
SD **Sunita Devi** MRN 66127 • Ward 6 • Bed 3	**Family Update** Medicine to Family	**Family (2)** Son, Sister	In progress	Call family
RK **Rakesh Kapoor** MRN 61009 • Day care • Bed 6	**Discharge planning** Medicine to Care Team	**Priya Nair** Case Manager	In progress	Specialist follow-up
PT **Pooja Tiwari** MRN 55902 • Ward 1 • Bed 4	**Handoff** Medicine to Nephrology	**Dr. A. Iyer** Intensivist	In progress	Assign owner

Showing 5 out of 22 care tasks

1 handoff needs you

AV Anita Verma
MRN 57122 • ER to ICU

Requested by **Dr. Shekhar Patel**

View Details
Accept Handoff

Recent handoffs

- Rajinder Singh
  ICU to Medicine handoff summary added.
  2 hours ago
- Sunita Devi
  Family meeting completed.
  4 hours ago
- Rakesh Kapoor
  Discharge documents shared with TPA.
  5 hours ago